

MEDINAZOLE PLUS CREAM
Econazole Nitrate 1%w/w, Triamcinolone Acetonide 0.1%w/w

Active Ingredients:
Econazole Nitrate 1%w/w
Triamcinolone Acetonide 0.1%w/w
Preservative: Chlorocresol 0.1%w/w
Presentation:
Tube of 15g box.
Product Description:
White smooth cream with faint smell.

Pharmacodynamics:
Econazole nitrate
Econazole nitrate acts by damaging fungal cell membranes, resulting in increased permeability. Sub-cellular membranes in the cytoplasm are damaged. The site of action is most probably the unsaturated fatty acid acyl moiety of membrane phospholipids.
Triamcinolone acetonide
Triamcinolone acetonide is primarily effective because of its anti-inflammatory, antipruritic and vasoconstrictive actions, characteristic of the topical corticosteroid class of drugs. The pharmacologic effects of the topical corticosteroids are well-known; however, the mechanisms of their dermatologic actions are unclear. Various laboratory methods, including vasoconstrictor assays, are used to compare and predict potencies and/or clinical efficacies of the topical corticosteroids. There is some evidence to suggest that a recognizable correlation exists between vasoconstrictor potency and therapeutic efficacy in man.

Pharmacokinetics:
Econazole Absorption
Systemic absorption of econazole is extremely low after topical application to the skin. Mean peak plasma/serum concentrations of econazole and/or its metabolites were observed 1 to 2 days after dose administration and were <1 ng/mL for the 2% Dermal cream applied to intact skin and 20 ng/mL for the 2% Dermal cream applied to stripped skin. Although most econazole remains on the skin surface (approximately 90%) after application of a 1% cream, concentrations of econazole that have been found in the stratum corneum exceed the minimum inhibitory concentration for dermatophytes, and inhibitory concentrations are achieved in the middermis.
Distribution
Econazole and/or its metabolites in the systemic circulation are extensively bound (>98%) to serum proteins.
Metabolism
Econazole that reaches the systemic circulation is extensively metabolized by oxidation of the imidazole ring, followed by O-dealkylation and glucuronidation.
Excretion
Econazole and its metabolites are eliminated in urine and feces in approximately equal amounts.
Triamcinolone Absorption
The extent of percutaneous absorption of topical corticosteroids is determined by many factors including the vehicle, the integrity of the epidermal barrier, and the use of occlusive dressings. Topical corticosteroids can be absorbed from normal intact skin. Inflammation and/or other disease processes in the skin increase percutaneous absorption. Occlusive dressings substantially increase the percutaneous absorption of topical corticosteroids. Thus, occlusive dressings may be a valuable therapeutic adjunct for treatment of resistant dermatoses.
Distribution
Once absorbed through the skin, topical corticosteroids are handled through pharmacokinetic pathways similar to systemically administered corticosteroids. Corticosteroids are bound to plasma proteins in varying degrees.
Metabolism
Corticosteroids are metabolized primarily in the liver.
Excretion
Corticosteroids are then excreted by the kidneys. Some of the topical corticosteroids and their metabolites are also excreted into the bile.

Indications:
Medinazole Plus Cream in all its dosages is indicated for the treatment of dermatomycoses complicated by inflammatory and/or pruritic manifestations of skin disorders.

Recommended Dose:
The duration of treatment with the Medinazole Plus Cream should be until the inflammatory symptoms subside but no longer than 2 weeks; after 2 weeks of therapy with Medinazole Plus Cream, continue therapy as needed with a preparation containing econazole or econazole nitrate alone.

Symptoms and Treatment of Overdose:
Medinazole Plus Cream is for cutaneous application only. Corticosteroids applied to the skin, including triamcinolone, can be absorbed in sufficient amounts to produce systemic effects.
Treatment:
In the event of accidental ingestion, treat symptomatically. If Medinazole Plus Cream is accidentally applied to the eyes, wash with clean water or saline and seek medical attention if symptoms persist.

Contraindications:
Medinazole Plus Cream is contraindicated in individuals who have shown hypersensitivity to any of its ingredients. Like any other dermatological preparation containing corticosteroids, Medinazole Plus Cream is contraindicated in specific skin conditions such as tuberculosis, varicella, herpes simplex or other viral infections of the skin, or fresh vaccination sites.

Warnings and Precautions:
For external use only. Avoid in contact with eyes, nose and mouth. Not to be taken. If symptoms persist, please consult a doctor.

Medinazole Plus Cream is not for ophthalmic or oral use.
If a reaction suggesting hypersensitivity or chemical irritation occurs, treatment should be discontinued.

Corticosteroids applied to the skin can be absorbed in sufficient amounts to produce systemic effects, including adrenal suppression. Systemic absorption may be increased by various factors such as application over a large skin surface area, application to damaged skin, application under occlusive skin dressings and prolonged duration of therapy.

Pediatric patients may demonstrate greater susceptibility to topical corticosteroid-induced HPA (hypothalamic-pituitary-adrenal) axis suppression and Cushing's syndrome than mature patients because of a higher ratio of skin surface area to body mass. Caution should be exercised when Medinazole Plus Cream is administered to pediatric patients and treatment should be discontinued if signs of HPA axis suppression or Cushing's syndrome occur.

Repeated application and/or prolonged application of topical corticosteroids in the periorbital region may induce cataracts, ocular hypertension, or increase the risk of glaucoma in patients.

Topical corticosteroids may lead to increased risk of dermatological superinfection or opportunistic infection.

Use in Pregnancy and Lactation:
Pregnancy

There are no adequate and well-controlled studies on adverse effects from the use of Medinazole Plus Cream in pregnant women, and no other relevant epidemiological data are available. Medinazole Plus Cream should be used in the first trimester of pregnancy only when the physician considers it essential to the welfare of the patient. Medinazole Plus Cream may be used during the second and third trimester if the potential benefit to the mother outweighs the possible risks to the fetus. Drugs of this class should not be used extensively in large amounts, over large skin surface areas, or for a prolonged period of time in pregnant patients. Studies in animals have shown reproductive toxicity. However, the risk in humans is unknown.

Econazole nitrate

Animal studies have shown reproductive toxicity. The significance of this finding in humans is unknown. Systemic absorption of econazole is low (<10%) after topical application to the intact skin in humans.

Triamcinolone acetonide

Animal studies have shown reproductive toxicity. Limited data in the literature indicate that up to 5% of topically applied triamcinolone to the skin is systemically absorbed in humans.

Breast-feeding

There are no adequate and well-controlled studies on the topical administration of Medinazole Plus Cream during breast-feeding. It is not known whether concomitant topical administration of Medinazole Plus Cream to the skin could result in sufficient systemic absorption to produce detectable quantities in breast milk in humans. Caution should be exercised when Medinazole Plus Cream is administered to breast-feeding mothers.

Econazole nitrate

Following oral administration of econazole nitrate to lactating rats, econazole and/or metabolites were excreted in milk and were found in nursing pups. It is not known whether cutaneous administration of topical econazole nitrate could result in sufficient systemic absorption of econazole to produce detectable quantities in breast milk in humans.

Triamcinolone acetonide

No studies in animals relevant to triamcinolone during lactation were identified. It is not known whether topical administration of triamcinolone to the skin could result in sufficient systemic absorption to produce detectable quantities in breast milk in humans.

Interactions with Other Medicaments:

Econazole is a known inhibitor CYP3A4/2C9. However, due to the limited systemic availability after cutaneous application, clinically relevant interactions are unlikely to occur, but have been reported for oral anticoagulants. In patients taking oral anticoagulants, such as warfarin or acenocoumarol, caution should be exercised and the anticoagulant effect should be monitored.

Side Effects:

Topical corticosteroids are associated with skin thinning and atrophy, striae, rosacea, perioral dermatitis, acne, telangiectasis, purpura, hypertrichosis, and delayed wound healing.

Occasionally, hypersensitivity, angioedema, contact dermatitis, skin atrophy, pruritus, skin exfoliation, skin striae, telangiectasia, erythema, application site pain, application site swelling are very rare.

Storage Condition:

Store below 30°C. Protect from light. Keep cap tightly closed.
Keep out of reach of children. Jauhkan daripada capaian kanak-kanak.

Shelf Life:

3 years from date manufacture
Registration No: **MAL00000000ACZ**

A.EWmNP4.01

Date of Revision: 14 August 2023

Product Registration Holder:

WORLD MEDICARE SUPPLIES SDN BHD
PT 5740(1st Floor), Jalan TS 2/1E, Taman Semarak 2, 71800 Nilai, Negeri Sembilan, Malaysia.

Manufacturer:

WINWA MEDICAL SDN.BHD
1952, Lorong I.K.S. Bukit Minyak 3, Taman I.K.S. Bukit Minyak,
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Size: 75mm x 190mm
■ Dark Blue (Pantone 2767U)
Arial MT, Font size: 4.5